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FOCUS GASTROENTEROLOGY

15 April 2026

Dr Noel Leon
Armstrong Street Medical Centre

Dear Dr Leon,

Re: Miss Julia Velakoulis
DOB: 1997-07-30
Address: 231 Canterbury Road, ST KILDA WEST VIC 3182

Summary

- **Gastroscopy with duodenal biopsies negative for coeliac disease.**
- **Borderline coeliac serology noted; for continuation of gluten-containing diet and monitoring.**
- **Functional bowel symptoms to be managed with fibre supplementation and gut-directed hypnotherapy.**
- **Family history of colorectal cancer does not necessitate increased surveillance.**
- **No action required by you.**

I had the pleasure of conducting a review with Julia following her gastroscopy and colonoscopy. Our primary focus was to correlate her recent clinical findings with the borderline serology results you had previously obtained.

As you are aware, Julia's serology showed a marginal elevation in one marker (21, with a reference range of 20), while other markers remained normal. During her gastroscopy, I took multiple biopsies from the small intestine to ensure a thorough assessment. I am pleased to report that the histology did not demonstrate any features of coeliac disease, nor were there any indeterminate signs of gluten enteropathy.

We discussed that while her current biopsies are negative, a borderline blood test means we cannot entirely exclude the future development of the condition. However, given the significant lifestyle burden of a gluten-free diet, I have advised Julia to continue consuming gluten. We await the

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320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

results of her HLA genetic testing, which was redirected to Australian Clinical Labs; a negative result would virtually exclude a diagnosis of coeliac disease.

Regarding her ongoing symptoms of flatulence and altered stool consistency, Julia describes her bowel movements as frequently "sticky." We discussed the role of the gut-brain axis and the impact of stress on gastrointestinal motility. I have suggested several conservative strategies to improve her stool consistency and regulation, including the introduction of a daily fibre supplement such as psyllium husk and the consumption of kiwi fruit. We also discussed dietary adjustments, such as substituting her current breakfast with porridge to provide better morning satiety and fibre.

Impression and Plan Julia appears to be experiencing functional bowel symptoms. In addition to the dietary measures mentioned above, I have recommended gut-directed hypnotherapy. I suggested she explore the Nerva application as a cost-effective and evidence-based method to manage the sensory signals between the gut and the brain.

We also reviewed her family history of colorectal cancer, which is limited to a paternal grandmother. I have reassured Julia that this does not place her in a high-risk category, and she does not require accelerated colonoscopic surveillance. I have advised her to continue a normal diet including gluten. If her symptoms significantly escalate in the future, we may consider repeating her serology. I will review her on a clinical needs basis.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist